

Sore Throat

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Sore throat is pain in the back of the throat.

A sore throat can be very painful and is usually worsened by swallowing.

Many people with a sore throat refuse to eat or drink. Sometimes pain is also felt in the ear because nerves to the back of the throat run very close to nerves from the ear.

Causes of Sore Throat

A sore throat usually results from infection (see table

Some Infectious Causes and Features of Sore Throat). The most common cause is

- Tonsillopharyngitis

Much less common but more serious causes of sore throat are

- Abscess
- Infection of the epiglottis (epiglottitis)
- Tumors

Abscess, epiglottitis, and tumors are of particular concern because they may block the airway.

Throat irritation and mild soreness can also be caused by dryness, irritants, postnasal drip from allergic rhinitis or chronic rhinosinusitis, gastroesophageal disease (GERD), and vocal strain (such as from yelling).

Tonsillopharyngitis

Tonsillopharyngitis is infection of the tonsils (patches of lymphoid tissue at the back of the throat) and the throat (pharynx). Doctors may use the term tonsillitis when the tonsils are particularly inflamed or the term pharyngitis when the tonsils are not particularly inflamed or when people who have no tonsils have a sore throat.

Tonsillopharyngitis is usually caused by a virus, typically one of the same viruses that causes the common cold. Most common colds begin with a mild sore throat. A less common viral cause is acute mononucleosis (caused by the Epstein-Barr virus), which occurs mainly in children and young adults. The coronavirus that causes COVID-19 and other coronaviruses can also cause a sore throat. Rarer still, sore throat can be part of the initial infection with human immunodeficiency virus (HIV) or due to chronic fungal infections in people with HIV infections, such as thrush.

About 10% of sore throats in adults (and slightly more in children) are caused by a type of *Streptococcus* bacteria (streptococci). Such streptococcal infections are often called strep throat. Strep throat is unusual in children younger than 2 years. If untreated, streptococcal infections can lead to serious complications such as rheumatic fever (now rare in developed countries), a kidney disorder called glomerulonephritis, pediatric autoimmune neuropsychiatric disorders associated with streptococcal infections [PANDAS], or an abscess.

Rare bacterial causes include anaerobic infections that may cause Lemierre syndrome (a frequent cause of acute recurrent sore throats), gonorrhea, and diphtheria (in countries with low vaccination rates).

Strep Throat



Strep throat (a bacterial infection) may cause a very red throat with white patches, but a sore throat caused by a virus may look the same. Thus, unless people clearly have just a cold, doctors usually do tests to diagnose strep throat.

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Abscess

A collection of pus ([abscess](#)) may form beneath or near one of the tonsils (peritonsillar abscess). The usual cause is a streptococcal infection that has spread from the tonsils into deeper tissue. In young children, an abscess can form in the tissue at the back of the throat ([retropharyngeal abscess](#)).

Epiglottitis

The epiglottis is a small flap of tissue that closes the entrance to the voice box and windpipe during swallowing. The epiglottis can become infected by certain bacteria ([epiglottitis](#)). This infection causes severe pain and swelling. The swelling can close off the windpipe, particularly in infants and children. Epiglottitis used to occur mainly in children and usually was caused by *Haemophilus influenzae* type B (HiB) bacteria. Now, in areas where most children are vaccinated against HiB, epiglottitis is quite rare in children, but HiB is still a cause in adults and unvaccinated children.

Evaluation of Sore Throat

Not every sore throat requires immediate evaluation by a doctor. The following information can help people decide whether a doctor's evaluation is needed and help them know what to expect during the evaluation.

Warning signs

In people with a sore throat, certain symptoms and characteristics are cause for concern. They include

- Squeaking sound when breathing in ([stridor](#))
- Any sign of difficulty breathing (particularly when children sit upright and lean forward, with their neck tilted back and jaw thrust forward to try to help them breathe better)
- Drooling
- Muffled, "hot potato" voice (speaking as if a hot object is being held in the mouth)
- Visible bulge in the back of the throat

When to see a doctor

People who have warning signs should go to the hospital right away.

People with a sore throat but no warning signs should call their doctor. People who have typical cold symptoms and mild to moderate discomfort may be advised to stay at home and [treat their symptoms](#) with over-the-counter (OTC) remedies. People with severe pain and/or other symptoms (such as fever, extreme fatigue, or a productive cough) typically should be seen right away.

What the doctor does

Doctors first ask questions about the person's symptoms and medical history and then do a physical examination. What doctors find during the history and physical examination helps them decide what, if any, tests need to be done (see table

[Some Infectious Causes and Features of Sore Throat](#)

During the medical history, doctors ask about the following:

- Symptoms of a runny nose, cough, and difficulty swallowing, speaking, or breathing

- Whether the person has had any general feeling of severe fatigue before the sore throat (suggesting mononucleosis)
- Whether the person has had a previous episode of mononucleosis (people rarely get mononucleosis twice)
- Whether people have any risk factors for gonorrhea (such as recent oral-genital sexual contact) or HIV infection (such as unprotected sex, multiple sex partners, or intravenous drug abuse)

During the physical examination, doctors focus on the nose and throat. However, if doctors suspect epiglottitis in children (because there are warning signs and no evidence of a cold), they do not examine the throat in their office because inserting a tongue depressor may cause a spasm that triggers complete airway blockage.

If epiglottitis is not suspected, doctors do the following:

- Look in the mouth to see whether the throat and/or tonsils are red and enlarged/swollen, whether there are white patches (exudate) on the tonsils, and whether there are any bulges suggesting an abscess
- Examine the neck for enlarged, tender lymph nodes
- Feel the abdomen for an enlarged spleen, which may occur in people with mononucleosis



Some Infectious Causes and Features of Sore Throat

Cause	Common Features*	Diagnostic Approach
<u>Viral tonsillopharyngitis</u> (infection of the tonsils and throat caused by a virus)	Usually mild to moderate throat pain and little or no fever Usually a runny nose and/or cough Throat and tonsils that are slightly to very red and that may be coated with a white discharge or pus Sometimes one or two enlarged lymph nodes in the neck	A doctor's examination alone
<u>Strep throat</u>	Often severe throat pain and fever Rarely a runny nose Cough is usually absent Often a very red throat and tonsils that are coated with a white discharge or pus Usually one or two tender, enlarged lymph nodes in the neck	Testing of a sample taken from the throat with a swab Sometimes throat culture in adults
<u>Mononucleosis</u> (caused by the Epstein-Barr virus)	Often moderate to severe throat pain, high fever, and constant fatigue but no cold symptoms	A blood test for antibodies to Epstein-Barr virus



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